

GLUMIC® — Semaglutide (GLP-1) Brand Plan: Market Entry Strategy

MBA Pharmaceutical Management — Summer Internship Project Report

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1. The Strategic Question

1.1 A Late Entrant into a Decelerating Category

India's GLP-1 receptor agonist market reached Rs 1,906 Cr on a MAT May 2026 basis, having grown 237% year-on-year after the semaglutide compound patent lapsed on 20 March 2026. That headline conceals the more important signal: monthly semaglutide injectable value growth collapsed from +50% to +6% between April and May 2026. The generic land-grab was already normalising onto a new base by the time this plan was written.

This report sets out the FY2026–27 brand plan for GLUMIC®, a once-weekly semaglutide injection pen (0.25 / 0.5 / 1.0 mg), authored during a summer marketing internship at Micro Labs Ltd. and delivered as a 37-slide audited strategy deck. The brief was deliberately hard: enter a category with more than 35 semaglutide brands from 17 companies, as a company ranked 18th in the Indian pharmaceutical market, without a GLP-1 heritage and without the field-force density of the incumbents.

The central judgement of the plan is that a late entrant cannot win this market on price, and should not try. Tirzepatide holds 63% of category value against semaglutide's 31%, and it holds that share by winning on weight-loss efficacy. The data publisher's own reading of the tirzepatide rebound — that it indicates “stickiness of superspeciality physicians to innovator molecules” — points to a market entering consolidation, where trust rather than discount determines the prescription.

Molecule	Value share (MAT '26)	YoY	Strategic read
Tirzepatide	63%	+2,524%	Category leader; wins on weight-loss efficacy (SURMOUNT-5)
Semaglutide	31%	+44%	35 brands / 17 companies; commoditising fast on price
Dulaglutide	4%	–17%	Declining; not a competitive threat
Liraglutide	2%	+40%	Small base; legacy positioning

Source: PharmaTrac MAT May 2026. Category value Rs 1,906 Cr.

2. Primary Research

2.1 Method

Rather than build the plan on secondary data alone, I ran two primary instruments across three Jaipur micro-markets. The first was a Retail Chemist Prescription Audit (RCPA) of 40 pharmacies, field-scored on

quality, prescriber linkage and potential. The second was a structured, signed questionnaire administered to 10 specialists across endocrinology, diabetology, internal medicine and gastroenterology. Both instruments are directional rather than statistically confirmatory at this sample size, and the plan states that limitation explicitly rather than overselling the base.

2.2 What the Field Data Changed

The field round did not decorate a pre-formed strategy; it overturned one. Two findings reshaped the plan.

Finding	Result	Consequence for the plan
GLP-1 actively dispensed	2 of 40 pharmacies	The territory is greenfield, not contested. The task is to build the category and prescriber habit, not to take share from an incumbent.
Cost cited as #1 adherence barrier	7 of 10 physicians	Affordability, not efficacy, is the binding constraint. The plan leads with a patient-support and adherence engine rather than conventional detailing volume.
Semaglutide rated best in practice	6 of 10 physicians	Molecule preference is winnable; the brand, not the molecule, is the open question.
Tirzepatide rated best	2 of 10 physicians	Cited reason was higher weight-loss efficacy — confirming the weight-loss buyer is not ours to win.
GLP-1 awareness among pharmacies	28 of 40	Awareness far outruns access: the gap is affordability and initiation, not education alone.

Source: Author's field research, Jaipur micro-markets. Instruments: 40-outlet RCPA; structured HCP questionnaire (n=10, signed). Directional pilot — not statistically confirmatory.

The greenfield finding is the one that matters most, and it is counter-intuitive. A category that grew 237% nationally was, at street level, barely being dispensed at all. That reframes the entire commercial question: share-of-voice tactics designed for a share war are the wrong instrument for a market where the category itself has not yet been built.

3. The Positioning Decision

3.1 Ceding the Weight-Loss Buyer

The plan makes one uncomfortable call explicitly: GLUMIC will not out-weight-loss tirzepatide, and should stop trying. Tirzepatide's superior weight-reduction data is the basis of its category leadership, and our own field survey confirmed that specialists who prefer it cite exactly that. Anchoring the brand's core claim on weight would place it second-best in a fight it cannot win, against a molecule with better data and a larger share of the recent surge.

The plan therefore cedes the cash-pay, BMI-led, aesthetically motivated patient by design, and concentrates the entire brand on a different buyer: the Type 2 diabetes patient with established or high cardiovascular risk who is paying out of pocket and for whom cost is the barrier to persistence. This is a segment tirzepatide cannot serve on cardiovascular evidence, and one the low-cost generics cannot serve on trust or service.

3.2 Segmentation, Targeting and Positioning

Segment	Profile	Priority	Prescriber tier
A — Primary	T2DM with high CV risk (HbA1c $\geq$ 8, established ASCVD)	Highest	Endocrinology + cardiology
B — High	Uncontrolled T2DM with obesity at step-up	High	Diabetology / internal medicine
C — Medium	Adherence-poor patients at step-up	Medium	Diabetology

Segment	Profile	Priority	Prescriber tier
D — High	Obesity with CV comorbidity, specialist-led	High	Bariatric / weight management
E — Reach	General medicine — pen awareness tier	Reach	Priority GPs

Segmentation dimensions: clinical severity (HbA1c, BMI, CV risk) × prescriber tier. Catchment sizing field-modelled; figures held in the internal annexure.

The resulting positioning statement is deliberately narrow: for cost-burdened T2DM patients with cardiovascular risk, GLUMIC is the semaglutide that pairs evidence-based cardiovascular protection with a price patients can sustain and a cold-chain they can trust — making metabolic protection achievable rather than aspirational. Positioning is a place in the prescriber's mind, not a discount; the plan holds a premium-value position and declines to chase the generic floor, using a secondary vial presentation to absorb price pressure so the hero pen never has to be discounted to clear stock.

4. Clinical Evidence and Claim Discipline

GLUMIC's clinical argument rests on molecule-level semaglutide evidence, and the plan is candid that this evidence is not proprietary — every semaglutide competitor can cite it. The edge is in detailing it first and best, not in owning it.

Trial	Population	Key result	Detailing use
SUSTAIN-6 (2016)	T2DM, high CV risk (N=3,297)	3-point MACE HR 0.74	Lead evidence for all T2DM detailing
SELECT (2023)	Overweight/obese with CVD, no T2DM (N=17,604)	MACE reduced ~20% (HR 0.80)	CV-risk-without-diabetes extension only
STEP 1	Weight, 68 weeks (N=1,961)	~15% body-weight reduction	Supporting, not the core claim
SUSTAIN programme	T2DM	HbA1c reduction up to ~1.5%	Glycaemic control support

Sources: SUSTAIN-6 (2016); SELECT (2023); STEP 1. Molecule-level semaglutide data.

The claim-discipline rule in the plan is strict: SELECT enrolled overweight and obese adults *without* Type 2 diabetes, so T2DM detailing must lead with SUSTAIN-6, and SELECT may be used only for the CV-risk-without-diabetes extension. Conflating the two would be caught immediately by a cardiologist, and the credibility cost of one over-claim in a specialist audience exceeds any short-term prescription gain.

5. From Insight to Commercial Model

5.1 The Adherence Engine

Because the field data identified cost-driven non-adherence as the binding barrier, the plan treats an injectable sold on persistence as a service rather than a product, and extends the marketing mix to the service Ps — People, Process and Physical Evidence — where trust is manufactured and where a price-led competitor cannot easily follow. The patient-support programme concentrates on the two points where the journey leaks hardest: initiation, where cost shock and injection fear kill the first prescription, and month-three persistence, where cost fatigue ends therapy.

Journey stage	Where value leaks	Programme response
Initiation	Cost shock and injection fear at first script	Affordability framing; pharmacist-led pen training at point of dispense
Titration	Early GI effects drive drop-off across dose steps	Titration support and side-effect guidance

Journey stage	Where value leaks	Programme response
Persistence	Cost fatigue by month three — the #1 barrier	Adherence diary; refill-gap flag triggering re-engagement
Advocacy	Stable patients unconverted into proof-points	KOL case-sharing; pharmacy anchoring

5.2 Cold-Chain as Brand Promise

In a market where Rajasthan summers routinely exceed 40°C, a validated 2–8°C cold-chain is not a logistics line item but the brand promise itself. The plan gates stockist selection on cold-storage capacity, places temperature data-loggers at every link with named accountability per node, and treats any excursion as a reputation crisis rather than a cost — quarantining the batch and replacing patient stock same-day. For a brand with no GLP-1 heritage, this is one of the few trust signals available at launch.

5.3 Frameworks and Regulatory Frame

The plan applies STP, the 4Ps extended to 7Ps, SWOT developed through a TOWS matrix into actionable strategy, KOL mapping, a perceptual value-versus-price map, and a risk register scored on likelihood and impact. It is built inside the Indian regulatory frame throughout: CDSCO approval scope and specialist-prescribing considerations, DPCO pricing exposure, Schedule H requirements, and UCPMP 2024 constraints on how any patient benefit may flow. A three-year commercial model in Excel tested viability on a patient-month accrual basis, with the financial detail held internally.

6. What the Project Demonstrates

The value of this project is not that the recommendation is clever in the abstract; it is that the field data was allowed to overrule the plan. The initial instinct for a late entrant in a hot category is to compete on share of voice and price. Forty pharmacies and ten physicians said otherwise — that the category was barely dispensed, and that cost, not persuasion, was ending therapy — and the strategy was rebuilt around them.

The plan's discipline is in what it declines to do: it does not claim the weight-loss segment it cannot win, does not match a price floor that would destroy the position, and does not stretch trial data past the population it enrolled. Those three refusals are the plan.

Competency	Demonstrated through
Market sizing and secondary analysis	IQVIA and PharmaTrac interpretation; class evolution and share dynamics
Primary research design	40-outlet RCPA and signed 10-physician survey; honest treatment of sample limits
Segmentation, targeting, positioning	Five-segment model with prescriber-tier mapping and a narrow positioning call
Marketing mix and portfolio strategy	4P/7P architecture; two-SKU ladder shielding the hero presentation
Commercial modelling	Three-year Excel model on patient-month accrual; scenario and payback logic
Regulatory and compliance literacy	CDSCO, DPCO, Schedule H and UCPMP 2024 applied throughout

Academic launch simulation prepared during a postgraduate summer internship. GLUMIC® is an illustrative brand. Market figures combine IQVIA and PharmaTrac data with the author's own field research. Internal financial projections, budget detail, and the identities of participating physicians and pharmacies are held in the internal annexure and are deliberately excluded from this public summary.